

COHERENCE SYSTEMS

Reaching the 95%, Measured.

The measurement instrument for the R95 Initiative — a discreet, no-PHI bridge that reaches the people the system was never designed to reach.

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WHERE THIS COMES FROM

A different way of working with AI

AI gets very good, very fast, at whatever you measure. Point it at the wrong target and it optimizes the wrong thing — efficiently. The fix wasn't a smarter algorithm. It was a better question: is everyone in the system better off, or just one number? We named that question **coherence**.

The thing that makes AI safe to work with is the same thing that makes a care system work: align everyone, and measure whether anyone is being left behind.

WHAT WE BUILD

Why that matters here

Coherence Systems builds the layer that keeps AI — and the systems it touches — honest about whether they're actually helping the person, not just hitting a target.

Reach

An instrument that can tell whether you're truly reaching the 95%.

Dignity

A patient-facing tool that meets people with dignity, not throughput.

Same principle, human scale.

THE GAP, IN YOUR NUMBERS

The philosophy is settled. The measurement isn't.

94.7%

of adults with SUD don't seek care
or believe they need it

SAMHSA NSDUH — adopted by SAPC R95

~3,000

drug-related overdose deaths in LA
County (2023)

LA County DPH, 2024 report

26%

of Service Helpline callers connect
to a treatment provider

SAPC SASH • Jul 2025–Mar 2026

These are the 95% — mid-collapse. Today's metrics can look excellent while none of them arrive.

Why good systems still miss the 95%

What we measure today

- **Units of service** — throughput, not reach
- **Completion rates** — the caseload that stayed
- **Billing & compliance** — what the system records

What they can't see

These are single-node metrics. A program can hit every one of them while the 95% never arrive — and while people are discharged for the very symptom we're meant to treat. The metric rewards throughput, so the system optimizes throughput. The person can be quietly left out of the math entirely.

The same failure, at every level

Look closely and it's always the same shape — the scarce human is spent on grind, so the work only they can do never happens.

Person

Hold times, dropped calls, a bed that filled yesterday — burning the moment of readiness.

Helpline

Nine agents, one in seven calls missed — the screening only an agent can do never happens.

Clinician

The scarcest resource in the field, buried in documentation — squeezing out the crisis care only they can give.

Analytics

The data team wrangling data instead of using it to fix the system's pain points.

Coherence: take the grind off the human at every level — and measure that everyone is better off.

THE MOVE

Put the person back in the math

Data for the organization



Data for the person

Our patented Key Coherence Indicators (KCI) framework makes the **beneficiary a measured participant in the system**. The moment you do that, any practice that hits its numbers by extracting from clients — abstinence-gated admission, discharge-on-relapse, toxicology used as punishment — shows up as a **measurable drop in system coherence**, not a hidden success. It is R95's philosophy, expressed as math.

The same provider, two ways of seeing

Today's view

Provider A admits 100 easy-to-serve clients and discharges 20 after a relapse.

Completion KPIs: **excellent.**

The relapse looks like a clean caseload. The 95% never appear in the data at all.

Under KCI

Those 20 relapse-discharges crater the clients' own trajectory and spike the friction at the boundary.

System coherence: **falls.**

The gaming becomes visible. Toxicology-as-punishment becomes a measurable coherence leak — not a quality signal.

Relapse is a symptom, not a failure. The math finally agrees with the medicine.

WHAT THE EVIDENCE SAYS

People hide to protect what recovery restores

The fears that keep people silent — 'I'll lose my job, my family, my reputation' — are often the very things treatment gives back.

65%

continuously abstinent at 6 months
(employed residential patients)

51%

still abstinent at 12 months



employment in recovery predicts
higher self-esteem, lower
depression, less family distress

Recovery Research Institute; JSAT (PMC3882309); PMC9130820

The secret turns out to be heavier than the help.

THE FIRST INSTRUMENT

The Bridge — a companion for the motivation window

1

Two doors

One for the person. One for the loved one who noticed first — because we see in others what we miss in ourselves.

2

Holds the progress

A save-point that survives relapse, dead batteries, lost signal.

3

Shows what's near

A private map of real LA County help — by area and need — one tap to the Helpline.

No PHI • No login • No procurement • Runs alongside RecoverLA • 42 CFR Part 2-safe

Try it now: holtsway.github.io/quiet-start

BUILT FOR YOUR SYSTEM, NOT AROUND IT

It rides on rails you already laid

- **R95 low-barrier design**

Meets people not yet ready for abstinence — your Focus Area, in their hand.

- **ASAM & MAT-aligned**

Routes toward medication and care; never gates on readiness.

- **BHIN 23-068 safe**

Adds zero documentation burden. No shadow records, no new forms.

- **42 CFR Part 2 posture**

Local-first, no-PHI — the strongest confidentiality posture for SUD data.

- **Maps to VBI dollars**

Supports the R95 incentive lines — 3-D, 3-F, 3-G and 3-H — directly.

- **Cost under pressure**

More R95 admissions at fewer staff-hours each — against a contracting budget.

THE ASK

Sixty days. No risk to your systems.

Scope

A small slice of RecoverLA traffic points to the Bridge. Nothing else changes.

Signal

A non-PHI, beneficiary-side coherence signal — the number SAPC has never had.

Success

A measurable lift in time-to-first-treatment, the event you already pay on.

No procurement. No PHI. No exposure to Sage. If it doesn't move the number, we shut it down with nothing lost.

Built on your existing infrastructure — no new systems, no new cost.

WHAT THIS OPENS

The same idea, across the system

The Bridge is the first of these. The same move — a specialized agent that gives a scarce human their time back — fits everywhere the system loses people.

Connection data, kept true

Your own SASH manual says the bed data isn't real-time and providers update it by hand. AI keeps it current — feeding your Network Adequacy Report instead of fighting it.

Meeting coherence

Record, synthesize, and track action items across meetings — surfacing gaps, overlaps, and priority conflicts, so the team meets less and aligns more.

Provider communication

Keep SAPC ↔ provider policy, guidance, and support coherent — and extend it to help providers run better.

Analytics, freed

Let your data team use the data to fix the system, instead of wrangling it.

We don't sell AI — we sell coherent implementation. That's the hard part, and it's the part we do.

**“The opposite of addiction
is not sobriety —
it is connection.”**

We built the instrument that measures it.

Coherence Systems LLC · SDVOSB · Reaching the 95%, Measured

Ask: 60-day pilot + a working session with your Chief of Data & Analytics.